

SEPSIS HOUR-1 BUNDLE

On suspicion of sepsis or septic shock, the synthetic hour-one bundle advises rapid measurement of serum lactate, drawing of blood cultures before initiating antibiotics when feasible, and prompt empiric broad-spectrum coverage. Crystalloid resuscitation at thirty milliliters per kilogram is suggested for hypotension or lactate at or above four millimoles per liter. Vasopressors should be initiated when mean arterial pressure remains below sixty-five millimeters of mercury despite adequate fluids.

Reassessment of perfusion is recommended within the first hour, with ongoing tracking of lactate clearance, capillary refill, and urine output. The bundle is designed to compress decision latency at the front of the resuscitation, not to replace clinical judgement.

ANTIMICROBIAL STEWARDSHIP

Synthetic guidance suggests that empiric coverage should be broad at presentation and narrowed within forty-eight to seventy-two hours as culture data return. Duration of therapy is typically seven to ten days for most uncomplicated bloodstream sources, with longer courses for endocarditis, osteomyelitis, or undrained foci.

Procalcitonin trends may inform discontinuation of therapy in clinically improved patients but should not be the sole driver of stop decisions.

VASOPRESSOR SELECTION

Norepinephrine is the suggested first-line vasopressor for septic shock in this synthetic guidance. Vasopressin may be added at a fixed low dose to reduce norepinephrine requirements. Epinephrine is reserved for patients with inadequate response to first-line agents or with concurrent myocardial dysfunction.